

The art of medicine

The fall of the schizophrenogenic mother

Can bad mothers and dysfunctional families cause young people to become schizophrenic? We know that, as recently as a generation ago, a large sector of Anglo-American psychiatry and clinical psychology believed that they could. Elite private hospitals offered patients with schizophrenia intensive forms of psychoanalytic therapy designed to uncover the trauma responsible for their suffering. Mothers of children with schizophrenia were given personality tests designed to discern what specific traits might be responsible for their children's distress; and family patterns of interaction were studied with an eye to unmasking the specific "crazy-making" dynamics.

Today, memories of that whole era make many people wince. Schizophrenia is now understood to be a brain disorder, best treated with medication. The psychiatric profession is appalled by the burden and pain that was once inflicted by telling families, and especially mothers, that they had literally driven their children crazy. That all said, there is a lot that remains imperfectly understood about how we got to the point we are at today. We are often told that, just when the psychogenic theories of schizophrenia had reached a peak of offensiveness and empirical vacuity, some new biological research came riding in, like the cavalry, and finally got things back on track. In fact, the historical record does not seem to bear this out.

Certainly, there was biological research that made a difference. In the early 1960s, investigators explored the possibility that schizophrenia was a biochemical disorder involving excess levels of dopamine (in part because drugs like chlorpromazine that reduced symptoms of schizophrenia also reduced available levels of dopamine in the brain). Other researchers began suggesting that certain neuroanatomical markers, such as enlarged ventricles, could reliably distinguish the normal brain from the schizophrenic brain. In the early 1970s, Seymour Kety published an influential adoption study that suggested a genetic basis to the disorder by showing, among other things, that children born to mothers with schizophrenia but raised by other people develop schizophrenia at the same rate as those reared by their biological mothers. Tellingly, though, much of this work was done during the heyday of the schizophrenogenic mother era, and thus co-existed with psychogenic approaches. Despite these advances in research, the question as to whether schizophrenia was a psychogenic or a biological disorder was considered unsettled right into the early 1980s. Indeed, in many textbooks and medical school courses, where psychoanalytic perspectives still dominated, the biological arguments were often given short shrift. I would suggest that things finally changed, not so much when psychiatry definitively won its case, but when the world changed around psychiatry and made psychogenic approaches to schizophrenia feel both disrespectful and impractical.

Several converging developments contributed to this process. The first was the growing politicisation of the psychogenic approach to schizophrenia during the 1970s. In 1972, the American feminist psychologist Phyllis Chesler wrote an influential book in which she pointed the finger of blame, not at the schizophrenogenic mother (whose reality she did not question), but at the patriarchal society that had created her. This woman, she said, was a victim of oppression and trauma no less than her child. In these same years, the Scottish radical psychiatrist Ronald Laing became famous for arguing that the families who created the bourgeois, one-dimensional consciousness that our society calls "normalcy" were no less sick than those responsible for the experience we call "schizophrenia". While a few radical therapists attempted to translate such political arguments into new kinds of therapeutic interventions, the main audience for these kinds of theories increasingly came to be, not members of the mental health professions, but consumers of left-wing counterculture ideologies.

Compounding the problem here was a second development: growing doubts, not so much about the credibility of the idea that mothers or families can cause schizophrenia (though there was some of that), but rather about the



Atlanta Constitution, (1978), Clifford H. Baldowski Editorial Cartoons. Courtesy of the Richard B Russell Library for Political Research and Studies, The University of Georgia Libraries

effectiveness of the two psychotherapeutic methods—psychoanalysis and family therapy—generally used to treat schizophrenia in the 1970s. In 1983, Thomas McGlashan, who had been a leader in the psychoanalytic approach to schizophrenia at Chestnut Lodge, a psychiatric hospital in Rockville, MD, USA, revealed the results of his analysis of the case records of some 446 patients. McGlashan's findings pointed clearly to the conclusion, he said, that this kind of psychotherapy was ineffective for such patients. In his words, declared at a meeting of some 500 clinicians: "The data are in. The experiment failed." This mattered. What was the point of entertaining elaborate claims about the psychogenic roots of schizophrenia if in the end the approach failed to offer any therapeutic purchase?

About that same time, in 1981, social workers Carol Anderson and Gerard Hogarty published outcome findings for a new approach to family therapy in schizophrenia: something they called psychoeducation. It turned out that, when families were approached sympathetically and taught skills for coping better with the profound emotional and interpersonal challenges of being the front-line caregivers of a child with schizophrenia, there was a reduction in relapse rates in the children themselves. In sharp contrast to the dominant family therapy approaches of the time, this therapy concentrated, not on seeing families as the cause of their children's illness, but on building alliances with them. Blaming as a practical approach—especially in the 1980s, a time when so many more families were directly involved in the care of such patients—did not work.

This takes me to the final, and what I think was the most important, development responsible for the demise of the schizophrenogenic mother and family: the emergence of a new social reality in which mothers and families increasingly functioned as the long-term caregivers of their children affected by mental illness. This situation in the USA was in turn created by deinstitutionalisation: the radical social experiment of the 1960s and 1970s that involved releasing large numbers of patients with schizophrenia from psychiatric hospitals. The reasoning was that these patients could now live and be treated in the community if they were properly medicated.

The reality turned out to be more complicated. Some patients went off their medication, or refused to take it. Many others ended up homeless or in prison. The lucky ones had parents who tried to step into the breach and figure out solutions. But by a rather cruel irony, many of these family caregivers were precisely the same parents who had previously been censured by the psychiatric profession. Soon, they began meeting to discuss their sense of frustration and anger. They discovered and built alliances with the biological wing of psychiatry, which told them that schizophrenia was a disease like any other, and no one's fault, least of all theirs. By 1979, in the USA, they had transformed themselves into a formidable national movement known as the National

Alliance on Mental Illness (NAMI), which began to demand better treatment and care for people with schizophrenia, and better accountability from the psychiatric leadership that had misled and hurt them. In the words of one of these parents, declared before the World Congress on Psychiatry in 1977: "We failed to understand why parents of a child with leukemia were treated with sympathy and understanding, while parents of a child with schizophrenia were treated with scorn and condemnation."

By the early 1980s, a new cultural orthodoxy on the nature of schizophrenia was being actively constructed. One cultural marker of this process can be seen, for example, in a 1984 US Public Broadcasting Service documentary called *Madness*, which was devoted to the idea that schizophrenia was a brain disease. The programme showed psychiatrists interacting with patients and interviews with sympathetic mothers involved in the founding of NAMI. Prominent in the show was E Fuller Torrey, a psychiatrist who, in 1982, had written the first handbook designed specifically to help, not patients, but families "survive" schizophrenia. Taken as a whole, the programme functioned less as a tutorial on the biological basis of schizophrenia and more as a kind of extended public apology to parents.

The parents deserved an apology. The ousting of the schizophrenogenic mother was sorely needed. Nevertheless, there may have been unintended costs that have resulted from the way in which we have banished her. In our own time, cultural and psychosocial approaches to schizophrenia have become, in the eyes of many, not just scientifically inappropriate, but morally unacceptable. Let me be clear: I am not denying the good evidence for seeing schizophrenia as a neurobiological disorder, and I am far from trying to excuse the harm that was caused by blaming families for the disorder. My goal is different. I believe we are recovering, as a culture, from a traumatic chapter in the history of psychiatry and that our bumpy road into the present day has left us with some implicit taboos about the kinds of conversations we feel we may and may not have about the disorder, especially in public forums. Fear of giving any energy to discredited models of family blaming means that, today, cultural and psychosocial questions relating to schizophrenia are rarely discussed. I consider it axiomatic that all human beings, even those with biological vulnerabilities that put them at risk of schizophrenia, are embedded in a cultural and interpersonal world that gets under their skin and affects them. I suspect that few biological researchers of schizophrenia really doubt this either. It is time to find new and better ways to talk out loud and in public about the ways in which this is so; ways that will serve patients and families alike.

Anne Harrington

Department of the History of Science, Harvard University, Science Center 371, Cambridge, MA 02138, USA
aharring@fas.harvard.edu

Further reading:

Hogarty, Gerard E. Does family psychoeducation have a future? *World Psychiatry* 2003; **21**: 29–30

Hopper K, Harrison G, Janca A, Sartorius N, eds. *Recovery from schizophrenia: an international perspective*. Oxford: Oxford University Press, 2007

Mehta S, Farina A. Is being "sick" really better? The effect of the disease view of mental disorder on stigma. *J Soc Clin Psychol* 1997; **16**: 405–19

Neill J. Whatever became of the schizophrenogenic mother? *Am J Psychother* 1990; **44**: 499–505

Saks E. *The center cannot hold: my journey through madness*. New York: Hyperion, 2007